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Division of Dockets Management (HFA-305)
Food and Drug Administration
5630 Fishers Lane, Rm. 1061
Rockville, MD 20852

Re: Docket No. FDA-2015-D-3719 for “Draft Guidances Relating to the Regulation of Human Cells, Tissues, and Cellular and Tissue-Based Products; Rescheduling of Public Hearing; Request for Comments”

Dear Sir or Madam:

My name is Scott James, and I am a vascular surgeon and the current Medical Director of the Wound and Hyperbaric Oxygen Center and Vein Center at Beth Israel Deaconess Hospital in Plymouth. I hold a medical degree from the University of New England College of Osteopathic Medicine and have over 12 years of clinical experience in wound healing and vascular surgery. Thank you for this opportunity to comment on the HCT/P Draft Guidance Documents.

I commend FDA for focusing on the need for greater clarity in the regulation of human cell and tissue products. The need is particularly great with respect to HCT/Ps intended as regenerative medicine therapies, an area that is driving new innovation and growth and holds much promise for patient treatments in the future as well as meeting unmet medical needs. However, this is also an area in need of greater regulatory attention to ensure the safety of patients and protection of public health.

Over the last ten years, an entire inadequately regulated industry of large-scale-manufactured biological products has sprung up under the cover of a minimalist regulatory scheme originally designed to oversee, without undue regulatory interference, the distribution of traditional organs and tissues for transplant. The widespread marketing of Section 361 allografts that do not meet the criteria of Section 1271.10 has been possible because the regulatory scheme leaves distributors of allograft products to make their own determinations as to whether their products qualify as Section 361 HCT/Ps. There are powerful financial incentives for these distributors to determine that their products can legally go to market under the Section 361 pathway and few, if any, incentives for them to determine that they require premarket review. Not surprisingly, then, allograft distributors almost always conclude that no premarket review is necessary for their products. As a result, we see a disturbing number of product promoted to health care providers like us for uses that FDA has never reviewed or approved, up to and including claims that these products are comparable or even superior to products that have faced rigorous FDA premarket review.

As a vascular surgeon, my own observations of these issues have occurred in the wound care and limb salvage areas. In these areas, there are a large number of tissue products being marketed

without robust evidence demonstrating their safety and effectiveness. The marketing claims for these products have not been appropriately substantiated and in some cases they are also being marketed for novel applications. The lack of premarket review over these products has sown confusion among payers, with the very real effect that patients' access to therapies that are proven to be safe and effective has become much more limited.

The patients that we see in our practice have devastating conditions, and the consequences of using treatments that are not backed by rigorous science can be disastrous. Our patients deserve to know that the therapies we give them have been proven to be both safe and effective. It's that simple. Section 361 simply ensures the safety of cells and tissues from an infectious disease standpoint. That's all it does. But preventing tissues from transmitting disease is just the beginning of determining whether a tissue or cells are safe and effective for indications that implicate complex biochemical processes to achieve an intended therapeutic effect. From the beginning, it was FDA's intention that HCT/Ps intended for complex interactions that fall outside "normal use" for a "conventional tissue" would place these products squarely in a higher risk category, meaning that they would be subject to premarket scrutiny and greater post-market controls. Allograft distributors who have taken advantage of the ability to self-designate their products as Section 361 HCT/Ps have thoroughly distorted the regulatory framework, to the detriment of patients.

In short, it is critical for the wellbeing of our patients that the FDA take consistent and definitive actions to bring HCT/Ps that are intended to interact with the body in complex ways, for example, in the manner of cell therapies, be subjected to the same degree of regulatory scrutiny as other biological products with complex mechanisms of action. The draft minimal manipulation and homologous use guidance documents are a critical first step in restoring the regulatory scheme and making it work as it was always intended to work. For that reason, I join other commenters in urging FDA to proceed with all possible speed.

Thank you once again for your time and attention to these comments.

Sincerely,

Scott James, D.O.